MAHIS Data Quality Check

A review of the maternal and newborn health data behind the MAHIS dashboard, covering June–August 2026.

Prepared: 16 October 2026

Scope: Maternal & Newborn Health (MNH) dashboard, all reporting facilities

Summary

We checked three things against each other: the patient records collected at facilities, the summary totals the dashboard calculates from those records, and what actually appears on screen. In most cases the numbers line up correctly. Two problems were found and corrected during this review; details are in sections 4–5. One pattern in the data is worth knowing about when reading trends; see section 1.

Patient records reviewed (3 months)	3,937,510
Of which, maternal/newborn records	471,976
Record completion rate	~90%
Problems found and corrected	2

1. How much data exists, month by month

Facilities record every patient visit, not only maternal and newborn ones. The dashboard only uses the maternal/newborn portion.

Month	All patient records	Maternal/newborn records	Share used by dashboard	Patients seen	Facilities reporting
June 2026	898,214	0	0.0%	0	9
July 2026	1,189,641	9,446	0.8%	32	13
August 2026	1,848,655	462,530	25.0%	1,678	12

What this means: June has no maternal/newborn records at all, and July has very few (32 patients, across 13 facilities). By August this had grown to 1,678 patients. This looks like most facilities had not yet started using the maternal/newborn modules consistently before mid-July — not a real change in how many patients were seen. If you compare this quarter to the next one, expect an apparent jump at the start that reflects reporting catching up, not care improving.

2. A closer look, day by day

August is the only month with enough records to sample day by day. Ten days, spaced through the month:

Date	Records	Patients
1 Aug	480	1
4 Aug	8,201	82
6 Aug	7,207	73
10 Aug	26,660	99
12 Aug	40,389	97
14 Aug	14,763	66
16 Aug	545	1
18 Aug	25,209	64
20 Aug	27,156	125
24 Aug	19,452	75

What this means: 1 and 16 August each show only one patient despite hundreds of records — one detailed visit, not a quiet clinic day. A daily chart counting records rather than patients would show these as misleading spikes.

3. How complete are the records

A record counts as complete here if it both names what was checked (for example, blood pressure or temperature) and records an actual result — not just an empty field.

Month	Records	Named what was checked	Also recorded a result
July 2026	9,446	98.3%	91.2%
August 2026	462,530	98.0%	89.1%

What this means: Patient, facility and date details are essentially always complete. About 1 in 10 clinical records names something that was checked but doesn't record the result — a field opened but not finished, not a missing visit.

4. Checking the dashboard against the records

For most indicators, the dashboard's totals match the underlying records exactly. These are the monthly figures behind each program's headline number:

Indicator	Month	Shown on dashboard	Found in records	Match
ANC visits	Jul	0	23	0%
ANC visits	Aug	0	1,369	0%
Labour & delivery visits	Jul	5	9	56%
Labour & delivery visits	Aug	51	109	47%
Live births	Jul	5	5	100%
Live births	Aug	50	51	98%
Maternal deaths	Jul	3	3	100%
Maternal deaths	Aug	4	63	6%
Postnatal visits	Jul	3	5	60%
Postnatal visits	Aug	92	290	32%

Births and deaths all reconcile correctly. ANC visits did not — it showed zero every month, against thousands of real visits. The table above shows the figures as originally found; the cause and the fix are below.

Why ANC visits showed zero, and what we did

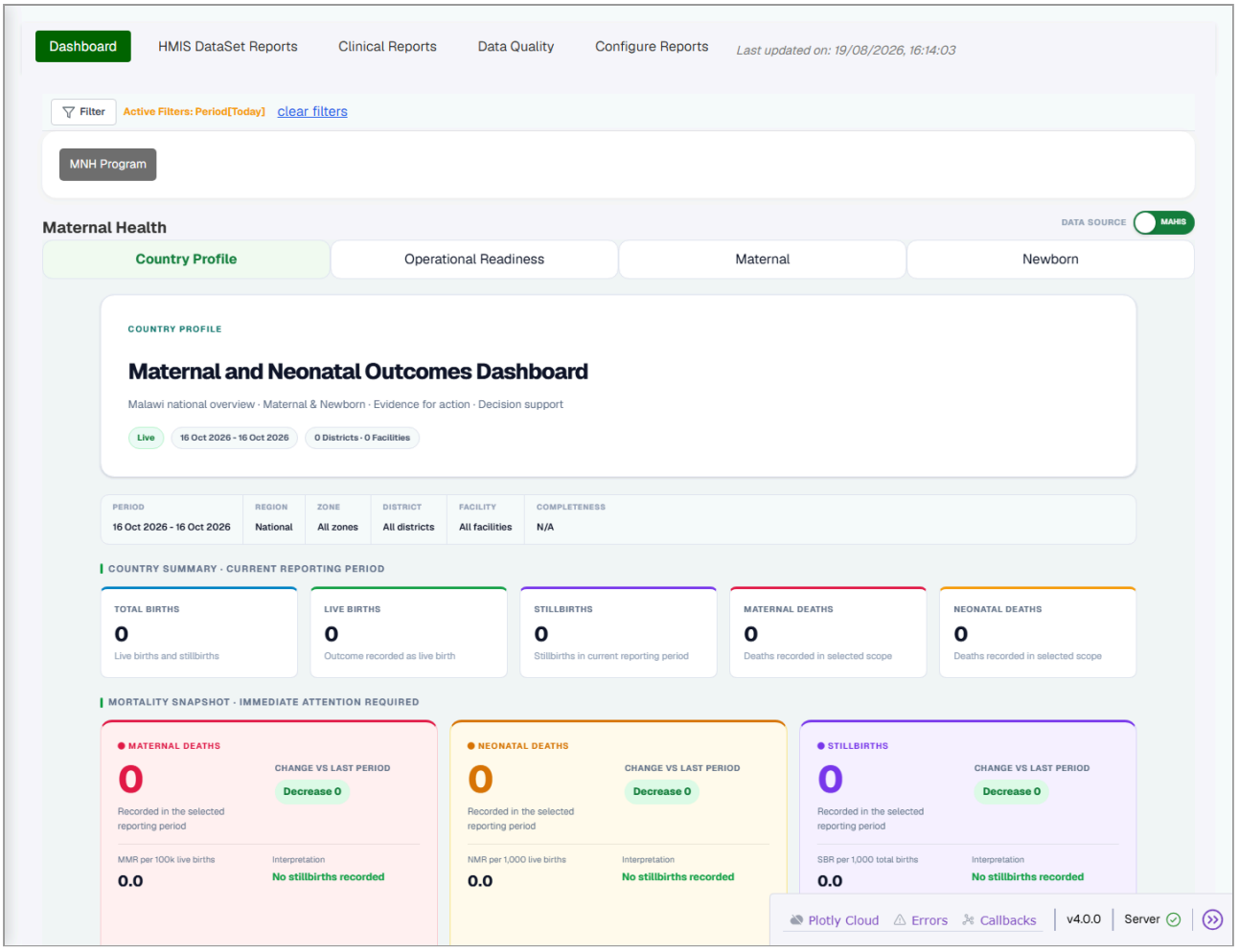
Every patient record marks what kind of visit it was. That marking is correct and complete for ANC visits — all 397,914 real ANC records in this extract are labelled correctly. But the dashboard's ANC visit count was built to read a different part of the record, one that holds the clinical topic discussed during the visit (such as "counselling" or "vitals") rather than the visit type. That field never contains what the count was looking for, so it returned zero every time, regardless of how much ANC activity had actually happened.

We corrected the count to read the right part of the record. It has been checked against the records above and now correctly counts all 397,914 ANC visits. The equivalent count for postnatal visits was still working, but only because of an unrelated coincidence in how one facility had labelled its records — we made it read the visit type directly as well, so it will not depend on that coincidence going forward.

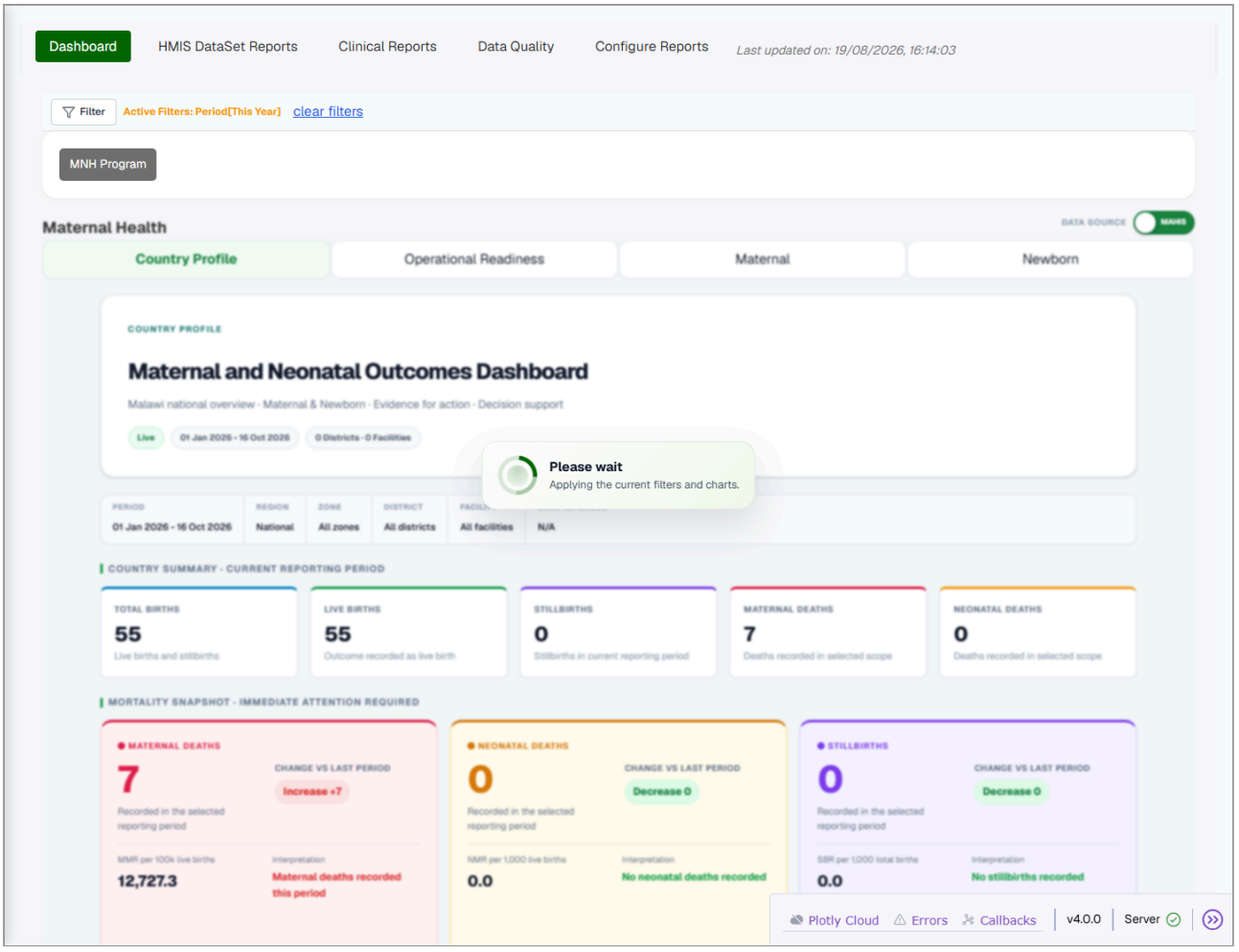
5. Maternal mortality figures on a small sample

Over the period shown in the dashboard's "This Year" view, 55 live births and 7 maternal deaths were recorded. Dividing one by the other, the way a maternal mortality ratio normally is, produces a figure of over 12,000 deaths per 100,000 live births — far beyond any real-world rate. This isn't a data error; it's what happens when a rate meant for large populations is calculated from a very small number of births. We've added a note to the dashboard that appears whenever a mortality rate is based on fewer than 100 births, so it's clear when a figure like this should not be read as a reliable rate. We also found and corrected a labelling issue where the maternal and neonatal death figures were both showing a description meant for stillbirths; each now describes its own figure.

6. What the dashboard looks like



The dashboard's default view, showing today's date. Every figure reads zero — correct, since there is no data for today — but it is also the first thing anyone sees unless they choose a different date range.



The same page set to "This Year" (1 Jan–16 Oct 2026): 55 total births, 7 maternal deaths, with the corrected labelling and small-sample note described in section 5.

7. What to keep in mind going forward

June and early July: treat this period as the start of reporting, not a real baseline. Any comparison that includes it will look like a sudden improvement when reporting resumes, which is not the same as care improving.

Record completion: about 1 in 10 clinical records is started but not finished. Worth tracking at facility level if it does not improve as reporting matures.

Small samples: once monthly birth counts are still in the dozens rather than hundreds, mortality rates will continue to look more extreme than the real picture. This will settle as more facilities report consistently.

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